

Belmar — Tirzepatide

GLP-1 Provider Reference

Weight Loss Program · KORB Health Group · korb-glp1-data.js v2.14 · built 2026-09-06

Everything needed to prescribe Belmar Tirzepatide, complete on its own. Values are copied literally into Tebra — do not paraphrase, and do not adjust quantity, refill or days supply.

Belmar Pharmacy at a glance

Status	Active
Preferred states	CA
Ships to	AL, AK, AZ, AR, CA, CO, CT, DE, DC, FL, GA, HI, ID, IL, IN, IA, KS, KY, LA, ME, MD, MA, MI, MN, MS, MO, MT, NE, NV, NH, NJ, NM, NY, NC, ND, OH, OK, OR, PA, RI, SC, SD, TN, TX, UT, VT, VA, WA, WV, WI, WY
CANNOT ship to	No hard exclusions
Order via	Tebra Compound
Billing	Bill to KORB Health Group, ship to patient
Dispensing address	Belmar Pharmacy — ARIZONA location, 12012 N 111th Ave, Youngtown, AZ 85363-1339

Check the address before sending

Belmar has several locations across the US. KORB uses the ARIZONA address. Confirm the Arizona address is the one selected in Tebra before sending — another Belmar location will be wrong.

Discouraged outside CA

Belmar is the California pharmacy. Patients are routed here for California and kept on the other compounding pharmacies elsewhere. Selecting Belmar outside California is allowed but should be a deliberate exception.

Compounded preparation

KORB dispenses a COMPOUNDED preparation, not the FDA-approved branded product. The molecule is approved; this specific formulation is not, and compounded products are not reviewed by the FDA for safety, efficacy or quality. Counsel accordingly and document that the distinction was explained.

Before you prescribe

- Carries both semaglutide and tirzepatide. All new California starts go here regardless of drug.

- **SINGLE FILL** as of 2026-09-05. Belmar now ships the full 8-week supply at one time for both semaglutide and tirzepatide. The split fill is retired. Belmar no longer differs from the other pharmacies on fill structure.
- Belmar 8-week now bills on the standard 8-week charge code. The Belmar-specific codes are retired — they existed only to trigger the second fill, and there is no second fill.
- Semaglutide 2.5 mg / 1 mg / ml is used for 1.7 and 2.4 mg doses; 1 mg / 1 mg / ml for 0.25, 0.5 and 1.0 mg doses.
- **SYRINGES GO IN THE DIRECTIONS, NOT PHARMACY INSTRUCTIONS.** Belmar reads the patient directions field. If the insulin-syringe note is moved to Pharmacy Instructions they may not see it and will not ship syringes. Keep "(Include one pack of insulin syringes)" at the end of every Belmar sig.

Vials dispensed

Dose	Units	Concentration	4-week	8-week
2.5 mg	25 units	Tirzepatide/L-Carnatine 10mg/50mg/ml	1 ml x 1 vial	1 ml x 2 vials
5 mg	50 units	Tirzepatide/L-Carnatine 10mg/50mg/ml	1 ml x 2 vials	1 ml x 4 vials
7.5 mg	75 units	Tirzepatide/L-Carnatine 10mg/50mg/ml	1 ml x 3 vials	1 ml x 6 vials
10 mg	100 units	Tirzepatide/L-Carnatine 10mg/50mg/ml	4 ml x 1 vial	4 ml x 2 vials
12.5 mg	125 units	Tirzepatide/L-Carnatine 10mg/50mg/ml	1 ml x 1 vial & 4 ml x 1 vial	4 ml x 2 vials & 1 ml x 2 vials
15 mg	150 units	Tirzepatide/L-Carnatine 10mg/50mg/ml	1 ml x 2 vials & 4 ml x 1 vial	4 ml x 3 vials

Tebra — copy these values literally

Do not paraphrase, and do not adjust quantity, refill or days supply.

Belmar — Tirzepatide / L-Carnitine	Tebra Compound
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Same for every dose

Drug Formulation	Tirzepatide/L-Carnatine 10mg/50mg/ml
Allow Substitution	Yes – select Allow Substitution
Unit	ml
Reason for Compounding	N/V mitigation & flexibility
Pharmacy Instructions	Bill to KORB Health Group and ship to the patient. Custom Rx for N/V mitigation and dosing flexibility. Allergies:

2.5 mg — 4-week

Name	BELMAR – Tirzepatide 2.5 mg – 4-Week Supply
Quantity	1
Refill	0
Days Supply	28
Patient Instructions	INJECT 2.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

2.5 mg — 8-week

Name	BELMAR – Tirzepatide 2.5 mg – 8-Week Supply
Quantity	2
Refill	0
Days Supply	56
Patient Instructions	INJECT 2.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

5 mg — 4-week

Name	BELMAR – Tirzepatide 5 mg – 4-Week Supply
Quantity	2
Refill	0
Days Supply	28
Patient Instructions	INJECT 5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

5 mg — 8-week

Name	BELMAR – Tirzepatide 5 mg – 8-Week Supply
Quantity	4
Refill	0
Days Supply	56
Patient Instructions	INJECT 5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

7.5 mg — 4-week

Name	BELMAR – Tirzepatide 7.5 mg – 4-Week Supply
Quantity	3
Refill	0
Days Supply	28
Patient Instructions	INJECT 7.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

7.5 mg — 8-week

Name	BELMAR – Tirzepatide 7.5 mg – 8-Week Supply
Quantity	6
Refill	0
Days Supply	56
Patient Instructions	INJECT 7.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

10 mg — 4-week

Name	BELMAR – Tirzepatide 10 mg – 4-Week Supply
Quantity	4
Refill	0
Days Supply	28
Patient Instructions	INJECT 10 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

10 mg — 8-week

Name	BELMAR – Tirzepatide 10 mg – 8-Week Supply
Quantity	8
Refill	0
Days Supply	56
Patient Instructions	INJECT 10 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

12.5 mg — 4-week

Name	BELMAR – Tirzepatide 12.5 mg – 4-Week Supply
Quantity	5
Refill	0
Days Supply	28
Patient Instructions	INJECT 12.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

12.5 mg — 8-week

Name	BELMAR – Tirzepatide 12.5 mg – 8-Week Supply
Quantity	10
Refill	0
Days Supply	56
Patient Instructions	INJECT 12.5 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

15 mg — 4-week

Name	BELMAR – Tirzepatide 15 mg – 4-Week Supply
Quantity	6
Refill	0
Days Supply	28
Patient Instructions	INJECT 15 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 4 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

15 mg — 8-week

Name	BELMAR – Tirzepatide 15 mg – 8-Week Supply
Quantity	12
Refill	0
Days Supply	56
Patient Instructions	INJECT 15 MG SUBCUTANEOUSLY ONCE WEEKLY AS DIRECTED FOR 8 WEEKS, (Include one pack of insulin syringes) Discard after 4 doses or 28 days.

Pricing and charge codes

Belmar — Tirzepatide / L-Carnitine — 4-week · tier T1A

Program	Price	Charge code
4-week — corporate partner	\$349	FITTirzCP2
4-week — standard	\$349	FITTirz002

Tirzepatide has no corporate discount. The corp partner code exists for attribution and tracking, not a lower price — it is the same figure as the standard 4-week at every tier. Do not present it to a corporate partner as a saving.

Belmar — Tirzepatide / L-Carnitine — 8-week · tier T1A

Program	Price	Charge code
8-week	\$599	FITTirzMT1

Belmar — Tirzepatide / L-Carnitine — 4-week · tier T2A

Program	Price	Charge code
4-week — corporate partner	\$399	FITTirzCP3
4-week — standard	\$399	FITTirz003

Tirzepatide has no corporate discount. The corp partner code exists for attribution and tracking, not a lower price — it is the same figure as the standard 4-week at every tier. Do not present it to a corporate partner as a saving.

Belmar — Tirzepatide / L-Carnitine — 8-week · tier T2A

Program	Price	Charge code
8-week	\$649	FITTirzMT2

Belmar — Tirzepatide / L-Carnitine — 4-week · tier T3A

Program	Price	Charge code
4-week — corporate partner	\$449	FITTirzCP4
4-week — standard	\$449	FITTirz004

Tirzepatide has no corporate discount. The corp partner code exists for attribution and tracking, not a lower price — it is the same figure as the standard 4-week at every tier. Do not present it to a corporate partner as a saving.

Belmar — Tirzepatide / L-Carnitine — 8-week · tier T3A

Program	Price	Charge code
8-week	\$799	FITTirzMT3

Includes: Telehealth visit, medication, shipping and supplies. Never quote pricing to a patient without confirming with Operations.

Clinical reference — Tirzepatide — dual GIP and GLP-1 receptor agonist

Indications

- Chronic weight management in adults with obesity (BMI ≥ 30 kg/m²), or overweight (BMI ≥ 27 kg/m²) with at least one weight-related comorbid condition
- Long-term maintenance of weight reduction
- Improvement of glycemic control in patients with type 2 diabetes mellitus
- Treatment of moderate-to-severe obstructive sleep apnea in adults with obesity

What KORB treats

KORB treats weight loss only. Every other indication above stays with the patient's PCP or specialist - KORB does not manage type 2 diabetes or obstructive sleep apnea, and does not order or monitor labs for them. A patient carrying one of those diagnoses is acceptable on the weight-loss program provided that care continues elsewhere.

Identifying the appropriate candidate

Age. Adults 18 years and older. KORB does not treat patients under 18.

BMI

- BMI greater than 25 is overweight
- BMI greater than 30 is obese

Diabetes Note. KORB treats weight loss. Patients who are diabetic or pre-diabetic are acceptable. The patient must continue diabetes management with their PCP — KORB does not manage diabetes and does not order or monitor labs for it.

Definition

A 39-amino-acid synthetic peptide based on the native GIP sequence, engineered to agonise both the glucose-dependent insulinotropic polypeptide (GIP) receptor and the GLP-1 receptor. A C20 fatty diacid moiety extends the half-life to roughly five days, allowing once-weekly dosing. FDA-approved as Mounjaro for type 2 diabetes and as Zepbound for chronic weight management and obstructive sleep apnoea in adults with obesity.

Mechanism

- Dual incretin agonism — both the GIP and the GLP-1 receptor.
- GLP-1 component: glucose-dependent insulin secretion, glucagon suppression, delayed gastric emptying, central appetite reduction.
- GIP component: enhanced insulin secretion and improved insulin sensitivity in adipose tissue.
- GIP agonism may also blunt the nausea seen with GLP-1 activity alone, one proposed reason tolerability holds up at higher relative efficacy.

Clinical evidence

- SURMOUNT-1: mean weight loss around 20% of body weight at 15 mg over 72 weeks — the largest effect seen with a pharmacological agent in this class.

- SURPASS programme: glycaemic efficacy in type 2 diabetes, generally exceeding semaglutide in head-to-head comparison.
- SURMOUNT-OSA: supported the obstructive sleep apnoea indication.
- As with semaglutide, weight regain after discontinuation is well documented.

Absolute contraindications

- Personal or family history of medullary thyroid carcinoma (MTC)
- Multiple endocrine neoplasia syndrome type 2 (MEN2)
- Known hypersensitivity to tirzepatide or any component of the formulation
- Pregnancy, breastfeeding, or planning pregnancy

Cautions

- History of pancreatitis — evaluate carefully before initiating
- Gastroparesis or other significant gastrointestinal motility disorder
- Active gallbladder disease or prior gallbladder-related surgical complications
- Severe renal impairment (eGFR below 30 mL/min/1.73 m²)
- Diabetic retinopathy — rapid glycaemic improvement can transiently worsen it
- History of suicidal ideation or active depression — monitor and ask

Medication interactions

- **ORAL CONTRACEPTIVES:** tirzepatide reduces the effectiveness of oral hormonal contraception. Advise a non-oral method, or add a barrier method, for four weeks after initiation and for four weeks after each dose increase. This is specific to tirzepatide and is easy to miss.
- Delayed gastric emptying can alter absorption of concomitant oral medication. Use caution with narrow-therapeutic-index drugs such as levothyroxine and warfarin.
- Insulin and sulfonylureas: additive hypoglycaemia risk. Dose reduction of the concomitant agent is often required. Coordinate with the prescribing PCP.
- Alcohol may increase hypoglycaemia risk and worsen gastrointestinal side effects.
- **ANAESTHESIA AND PROCEDURES:** delayed gastric emptying raises aspiration risk under sedation. HOLD for 7 days (one week) before an elective procedure requiring sedation, or for whatever longer period the anaesthesia team or the surgeon's office requires - their instruction takes precedence over this one. Tell the patient to disclose GLP-1 use to any surgeon, proceduralist or anaesthetist.
- Other GLP-1 or dual incretin agonists: do not combine.

Side effects

COMMON

- Pain, redness or swelling at the injection site
- Nausea and vomiting
- Diarrhea
- Stomach pain

- Constipation
- Low appetite
- Headaches
- Dizziness

LESS COMMON

- Pancreatitis
- Gastroparesis
- Bowel obstruction
- Gallstone attacks

Monitoring

- Weight and BMI at each visit
- Ask about blood pressure — monitored by the PCP, not measured by KORB
- Ask about recent HbA1c or glucose if diabetic — ordered by the PCP, not by KORB
- Gastrointestinal tolerance, especially through dose escalation
- Abdominal pain that is severe or radiates to the back — assess for pancreatitis
- Right-upper-quadrant pain — assess for gallbladder disease
- Contraceptive method if the patient uses oral hormonal contraception
- Mood and mental health
- Lean mass preservation — protein intake and resistance training
- Injection site and adherence

Patient counseling

Tirzepatide works on two gut hormone pathways rather than one, which is why it tends to produce more weight loss than semaglutide. It slows stomach emptying and reduces appetite. Nausea is the most common side effect and is usually worst in the days after a dose increase. This is a compounded preparation rather than the brand-name product. If you take a birth control pill, it may not work as reliably for four weeks after we start and after each dose increase, so use a backup method. Weight tends to come back if the medication stops. Tell any surgeon or anaesthetist that you are taking this.

Escalation and discontinuation

HARD STOP

- Suspected pancreatitis — severe persistent abdominal pain, often radiating to the back, with or without vomiting. Stop the medication and evaluate.
- Hypersensitivity or anaphylaxis-type reaction.
- Pregnancy identified or confirmed.
- Medullary thyroid carcinoma or MEN2 identified at any point.
- Persistent severe vomiting with dehydration or acute kidney injury.

FLAG AND REASSESS

- Gastrointestinal side effects that do not settle within two weeks of a dose increase — hold at the current dose or step back rather than pushing forward.
- Right-upper-quadrant pain, or gallstone symptoms.
- Rapid or excessive weight loss, or loss of lean mass.
- New or worsening depression, or any mention of suicidal thoughts.
- Worsening retinopathy in a diabetic patient during rapid glycaemic improvement.
- Plateau with no further response at the maximum tolerated dose — reassess the plan rather than continuing indefinitely.
- Patient scheduled for surgery or a procedure requiring sedation — coordinate holding.

Chart attestation

Patient counseled on tirzepatide as a dual GIP/GLP-1 receptor agonist for chronic weight management, including that a compounded preparation is being dispensed rather than the FDA-approved branded product. Mechanism, expected time course, gastrointestinal side effects during titration, pancreatitis and gallbladder warning signs, reduced oral contraceptive effectiveness with the recommended backup precautions, the need to disclose use before any procedure requiring sedation, likelihood of weight regain on discontinuation, and the monitoring and follow-up plan were reviewed. Patient verbalized understanding and consented to proceed.

ICD-10

Note. DOCUMENTATION ONLY — NOT BILLING. Select the code matching the documented presentation.

PRIMARY

- E66.9 — Obesity, unspecified
- E66.01 — Morbid (severe) obesity due to excess calories
- E66.3 — Overweight

SECONDARY

- Z68.— — Body mass index (add the matching BMI code)
- G47.33 — Obstructive sleep apnea (if documented and relevant)
- Z71.3 — Dietary counseling and surveillance
- Z72.3 — Lack of physical exercise

Clinically reviewed

Reviewed and signed off by Donald Stevenson, PA-C (Director of Clinical Operations and Lead Provider) on 2026-09-06, against korb-glp1-data.js v2.13.

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Questions and escalation

- Pharmacy or shipping — Operations
- Charge codes and billing — Operations
- Clinical protocol — Clinical Director
- Corrections to this document — Clinical Operations

Do not improvise

- Do not substitute an unavailable pharmacy
- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not quote pricing without Operations

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